



February 20, 2026

Department of Health and Human Services Office of the Deputy Secretary
HHS Health Sector AI RFI
Submitted electronically via [regulations.gov](https://www.regulations.gov)

Re: HHS Health Sector AI RFI

To Whom It May Concern:

The National Council for Prescription Drug Programs (NCPDP) is a not-for-profit American National Standards Institute (ANSI) Accredited Standards Developer (ASD) consisting of more than 1,300 members representing entities including, but not limited to, claims processors, data management and analysis vendors, federal and state government agencies, insurers, intermediaries, pharmaceutical manufacturers, pharmacies, pharmacy benefit managers, professional services organizations, software and system vendors and other parties interested in electronic standardization within the pharmacy services sector of the healthcare industry. NCPDP provides a forum wherein our diverse membership can develop business solutions, including ANSI-accredited standards and guidance for promoting information exchanges related to medications, supplies and services within the healthcare system.

NCPDP would like to thank the Department of Health and Human Services (HHS) for the opportunity to comment on the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care. NCPDP supports HHS' focus to establish a forward-leaning, industry-supportive and secure approach to accelerate the adoption and use of AI as part of clinical care.

While NCPDP has no direct comment on HHS' approach to regulation and reimbursement, NCPDP believes the continued funding of the Trusted Exchange Framework and Common Agreement (TEFCA) is an important part of research and development. The need to improve and enhance data exchange among all health care participants is critical to improve patient outcomes, reduce administrative burden and utilize emerging tools, such as AI. Comments on the specific questions can be found below.

1. **HHS Question:** *What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?*
 - a. **NCPDP Comment:** One of the potential barriers to innovation and AI adoption is the slow pace at which standards for industry use are named in regulations, such as those named in Medicare Modernization Act (MMA), Health Insurance Portability and Accountability Act (HIPAA) and others. Greater allowance of the industry to move standards forward more quickly would result in greater innovation in AI. Instead of requiring a specific version of a standard, applying a minimum standard version expectation could result in faster updates to processes utilizing the standards and, therefore, expedited innovation available for use with and for AI tools. The industry could adopt new features without government regulations adopting new versions resulting in greater support for AI innovation. To facilitate faster adoption of these standards, Medicare Part D and HIPAA regulations could adopt a process similar to ASTP SVAP.

Another barrier to AI adoption in health care is the degree of transparency within AI tools. As an example, the use of AI when renewing a prescription should be disclosed and documented, ensuring transparency to the pharmacist in case there are questions or concerns.



2. **HHS Question:** *What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.*
 - a. **NCPDP Comment:** As stated in Question 1, the use of AI tools in clinical care needs to be transparent, disclosed and documented throughout the care continuum to allow for full transparency and accountability. New policies designed to enforce this transparency need to be in place for clinicians and patients to trust those health care processes using AI tools. Regulations are needed to determine who is responsible when AI generates incorrect clinical guidance, how AI documentation should be stored to protect clinicians and safe-harbor provisions for clinicians and payers using CMS-approved, validated AI tools. Additionally, investing in AI literacy both pre- and post-entering the workforce is key to sustaining and developing AI tools and usage in clinical care. Literacy could be achieved through multiple channels including funding for creating national AI competency standards for health care providers, grants for operational AI training focusing on medication therapy management, claims resolution and workflow optimization and working with national education accreditation organizations to develop AI curriculum for students entering health care.
3. **HHS Question:** *For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?*
 - a. **NCPDP Comment:** Consent for the use of AI, transparency, accountability, product liability and regulatory authority are all issues that need to be resolved at a federal level to avoid a patchwork of state regulations. NCPDP recommends HHS convene expert panels, including representation from patient advocacy groups and professional organizations from across the legal and clinical spectrum of health care, to address these issues.
4. **HHS Question:** *How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?*
 - a. **NCPDP Comment:** NCPDP welcomes and encourages HHS and other governmental organizations to more actively participate in standards development to promote innovative and effective AI use to support clinical care.
5. **HHS Question:** *Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.*
 - a. **NCPDP Comment:** Enhancing interoperability leads to more robust AI training and usage. Through broader interoperability, clinical data available for training AI models on health care will be expanded with the expectation of creating better models and thereby enhancing patient care. As participation in TECCA increases, including pharmacy participation, expansion in interoperability of the care team should be a natural outcome.

NCPDP looks forward to serving as a trusted partner and resource as HHS works to create policies to ease health data exchange and promote innovation of AI in clinical care. NCPDP welcomes the opportunity to engage further, share our expertise and collaborate with HHS to make these goals a reality.

For direct inquiries or questions related to this letter, please contact:

National Council for Prescription Drug Programs, Inc.
9240 E Raintree Dr, Scottsdale, AZ 85260
480-477-1000 | www.ncpdp.org



Paul Wilson
Technical Analyst, Standards Development
NCPDP
standards@ncdpd.org

Respectfully,

A handwritten signature in black ink that reads 'Lee Ann C. Stember'.

Lee Ann C. Stember
President & CEO
National Council for Prescription Drug Programs (NCPDP)